

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

2

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°018/MVB_01/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (24)	ITURI : Aru, Aungba, Bambu, Damas, Bunia, Gety, Kilo, Komanda, Lita, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rwampara and Logo NORTH KIVU : Butembo, Goma, Oicha, Kalunguta, Katwa, Kyondo and Beni SOUTH KIVU : Miti-Murhesa
Reporting date	June 1, 2026
Publication date	June 2, 2026


 Total
confirmed cases

 Total deaths
among
confirmed cases

 Suspected cases
in progress
investigation

 Patients in
isolation -
hospitalization


Healed


 Contact tracing
rate in Ituri

*Data currently being harmonized

1. HIGHLIGHTS

- **Twenty-three (23) new confirmed cases** notified for the day of June 1, 2026, distributed in the health zones of **Mongbwalu (11), Bunia (6), Rwampara (3), Mambasa (2), Komanda (1).**
- A new health zone affected, namely that of **Mambasa.**

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons.

The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) epidemic, which was then raging in North Kivu.

The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

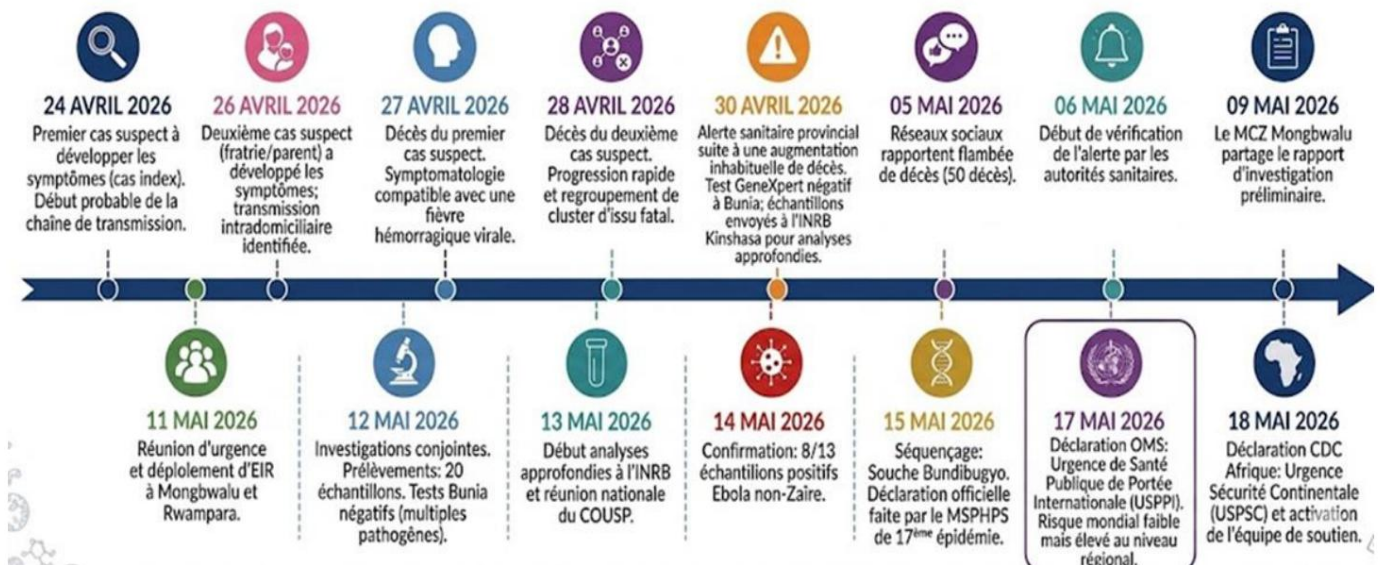


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial Distribution

Province	Case confirmed	Death (confirmed)	Lethality	Health zones touched	New cases (24h)
Ituri	322	46	14.3%	16 out of 36 (44.4%)	23
North Kivu	19	13*	68.4%	7 out of 34 (20.6%)	1 out
South Kivu	3	1	33.3%	of 34 (2.9%)	0
Total	344	60	17.4%	24 out of 104 (23.1%)	23

*Update note: North Kivu data has been revised based on updated reports received (June 1, 2026).

3.2 Affected health zones

- **Ituri (16 health zones):** Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rwampara.
- **North Kivu (7 health zones):** Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha.
- **South Kivu (1 health zone):** Miti-Murhesa.

3.3 Active foci and transmission dynamics

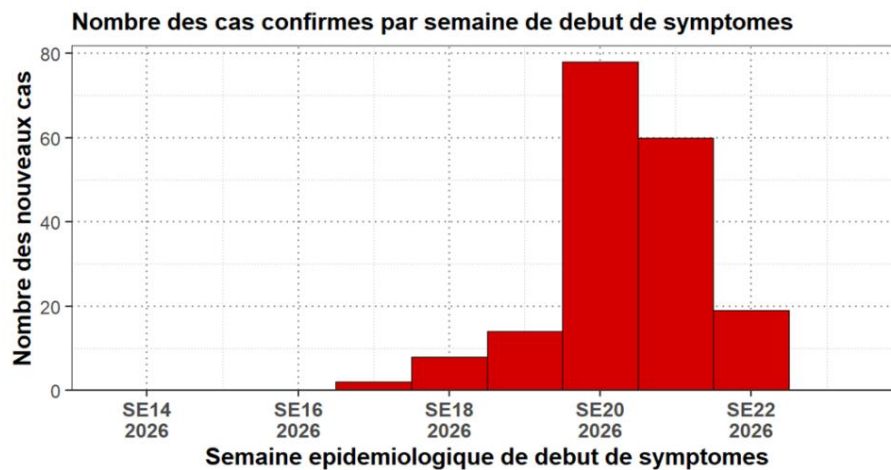
The **Ituri** province remains the national epicenter with **322 confirmed cases** (93.6% of the national total), **16 health zones affected**, and **23 new cases in 24 hours**. The case fatality rate there is **14.3%** (46/322), significantly lower than in other provinces, possibly linked to better access to care in Ebola Treatment Centers (ETCs) and the accumulated experience of the response teams since the beginning of the 17th epidemic.

North Kivu province presents a distinct epidemiological situation with **19 confirmed cases**, but a **case fatality rate of 68.4%** (13/19), the highest of the three provinces. This rate reflects several factors: delays in treatment, escapes from healthcare facilities (3 cases, one of whom was found deceased), community deaths, and difficulties accessing Ebola Treatment Centers (ETCs). Transmission affects 7 out of 34 **health zones**.

The main areas affected were Katwa (7 cases) and Beni (5 cases). No new confirmed cases were reported in the last 24 hours, but **30 new suspected cases** indicate ongoing active case finding. The **contact tracing rate of 59%** (510 contacts, 301 visits) is the main risk factor for continued transmission. Three areas (Kalunguta, Kyondo, and Oicha) reported **no contact visits** (0%).

South Kivu province remains a low-intensity hotspot with **3 confirmed cases** (all in the Miti-Murhesa health zone), **1 death**, and a **case fatality rate of 33.3 %**. No new cases have been confirmed in the last 24 hours. The **95% contact tracing rate** (142/150) is exemplary and represents the best defense against the geographic spread. An INRB mobile laboratory has been set up in Miti-Murhesa. Ten health zones are under surveillance due to suspected cases.

3.4 Time Analysis



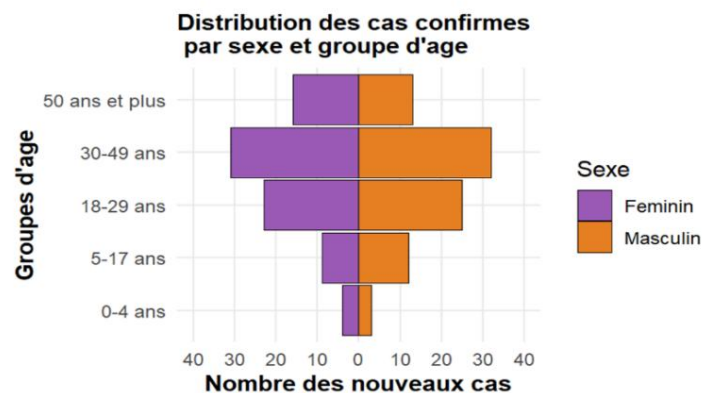
Source : DHIS2 Tracker - Riposte MVE, mai 2026. Données incluses dans l'analyse : n cas confirmés = 181.

Although the data is not complete, the current evolution of the disease shows a decrease in community transmission

3.5 Demographic Analysis

The **18-49** age group is the most affected, with a **male predominance** among the confirmed cases (approximately 55% male).

- The extreme age groups (children from 0 to 4 years old and adults 50 years and over) have a lower proportion of cases, although their higher risk of lethality requires special attention.

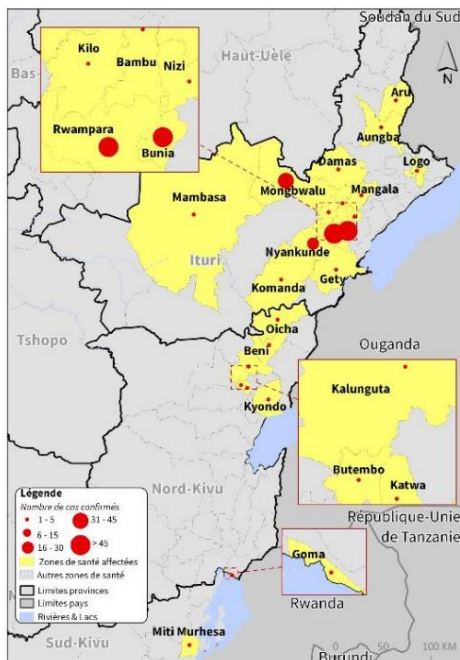


Source : DHIS2 Tracker - Riposte MVE, mai 2026.
: n cas confirmés avec âge et sexe renseignés = 168.

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 1, 2026 (*under consolidation*)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI			
Bunia	80	8	10.0
Rwampara	65	13	20.0
Mongbwalu	47	10	21.3
Nyankunde	19	1	5.3
Bamboo	2	2	100.0
Aru	2		50.0
Kilo	2	10	0.0
Nizi	2	0	0.0
Mangala		0	0.0
Damascus	11	0	0.0
Aungba	1	0	0.0
Gety		0	0.0
Komanda	11	0	0.0
Lita	1	0	0.0
Logo		0	100
Mambasa	12	1	50
Other ZS	94	10	10.6
(undistributed data)			
Ituri subtotal	322	46	14.3
NORTH KIVU			
Katwa	7	5	71.4
Blessed	5	3	60.0
Butembo	2	2	100.0
Oicha	2	2	100.0
Kalunguta	1	1	100.0
Kyondo		0	0.0
Goma	11	0	0.0
North Kivu subtotal	19	13	68.4
SOUTH KIVU			
Miti-Murhesa	3	1	33.3
South Kivu subtotal	3	1	33.3
TOTAL	344	60	17.4

3.6 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 1, 2026

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- Holding a meeting with experts for in-depth analysis to guide the response.
- Supporting the pillars on the use of picture boxes during relay briefings community.
- Deployment of response actors in health zones and needs assessment additional factors related to the dynamics of the epidemic.
- Holding an orientation meeting with the Director General of INSP, IM and IMA and the COUSP coordinators.
- Interview with the IM and the operations manager on modeling.

4.2 Epidemiological surveillance

Alert investigations

Table 2. Management of epidemiological alerts (24h), 17th MVE outbreak in Bundibugyo, by province, as of June 1, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Alerts raised	137			354
Alerts investigated	89			201
Investigation rate	65.0%	212,109	5 3	56.8%
Alerts confirmed	85	51.4% 17	60.0% 1	103

- Briefing of IT staff on community-based surveillance, Ebola surveillance in 6 health zones Ituri province (Aru, Nyankunde, Rwampara, Mongbwalu, Bambu, Bunia).

Contact tracking

Table 3. Follow-up of contacts of confirmed MVE Bundibugyo cases by province, Democratic Republic of Congo, as of June 1, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	3,350	1,317	39.3
North Kivu	510		59.0
South Kivu	150	301,142	95.0
Total	4010	1,760	43.9

Challenges in surveillance

- Community resistance in the Nizi and Bunia health zone for the listing and monitoring of contacts around confirmed cases.
- In-depth investigation of confirmed cases from May 31, 2026 in the ZS.

Point of Entry and Point of Control (PoE/PoC) Surveillance

- Training of frontline PoE/PoC providers on the minimum package of activities during the MVE epidemic response.
- Provision of tools and equipment to the 2 targeted PoCs (Marabo and Irumu on the axis Bunia-Komanda).



Training of frontline service providers on PoE/PoC based on the minimum package of activities

PoE/PoC Indicator	Value (24h)	Rate
Travelers who have used PoE/PoC	18,153	N / A
Screened travelers	17900	98.6%
Travelers who have washed their hands	17192	98.8%
Aware travelers	17442	96.0%
Listed contacts	1	N / A
Alerts raised	1	100.0%

4.3 Laboratory

Laboratory indicators (24 hours from June 1, 2026)

Indicator	Value	Analysis
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Samples collected	76	sustained activity
Samples analyzed	76	100.0% of those collected
Positive samples	23	Positivity rate 30.3%
Samples currently being analyzed	0	Delay in analytical capacity
Positive samples from deceased suspected cases	8	Transmission before confirmed death

4.4 Infection Prevention and Control (IPC)

Reported incident : Members of the Red Cross Team carrying out Safe and Dignified Burials (SDB) were attacked and beaten at the cemetery.

Actions by health zone

Health Zone	Actions taken
	Briefing of the authorities of the Gabriel School Group (Ring CH SOFEPADI) on hand washing.
Bunia	Briefing of 6 healthcare providers at CS Lembabo on triage principles and the implementation of the surveillance ring around the structure and provision of Thermoflash.
Nyankunde	Briefing 16 healthcare providers (10 EDS, 4 Supervisors and 2 IT) on standard precautions, wearing and removing PPE. 5 EDS carried out on 5 alerts (4 at HGR and 1 at AS Birinyama).
Rwampara	Decontamination of the 4 health facilities after their temporary closures (PS Espoir, PS Tumaini, PS Uponyadji and PS la Solution) and decontamination of the Maternity Ward of Clinique Elikya Mother-Child).
Bamboo	Installation of 37 (labeled) bins for medical waste in the various departments of the HGR.
Aru	Briefing of 21 healthcare providers at the BCZ on the donning and doffing of PPE and standard precautions, and support from the HGR

4.5 Holistic care

Table 4. Patient movement in healthcare facilities, 17th Ebola virus disease outbreak (Bundibugyo), Democratic Republic of Congo, as of June 1, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients present in bed (end of day D-1)	147	8	5	160
Admissions (24h)				
• PPL Admissions	2	0	0	2
• Other admissions	19	0	1	20
Total admissions	21	0	1	22
Exits (24h)				
• Healed	0	0	0	0
• Suspected deaths •	6	0	0	6
Confirmed deaths	1	0	0	1
• No cases •	2	0	2	4
Confirmed escapees	1	3	0	4
Total outflows	10	3	2	15
Patients in isolation (end of day) • New	158	8	7	173
confirmed cases (NC) • Previous	13	5	2	20
confirmed cases (AC) • Suspected	21	0	0	21
cases • Suspected	122	3	3	128
children < 5 years old*	2	0	2	4

*Children under 5 years old are counted both in the "Suspected cases" line and in the dedicated line.

As of June 1, 2026, **173 patients** were receiving care in healthcare facilities across the three provinces affected by the 17th Ebola virus disease outbreak (Bundibugyo species). The active caseload consisted of **41 confirmed cases** (20 new cases and 21 previously confirmed cases still hospitalized) and **132 suspected cases**.

diagnostic investigation is underway.

Ituri accounts for 91% of hospitalized patients (158/171), primarily in the Bunia General Referral Hospital (26), the Rwampara General Referral Hospital (22), and the Medical Center (23). The province recorded 21 new admissions and 10 discharges in the last 24 hours, including 7 deaths (6 suspected and 1 confirmed), with no recoveries.

North Kivu follows with 8 patients in isolation (5 confirmed cases and 3 suspected cases). No new admissions or deaths in healthcare facilities were reported in the last 24 hours. Three confirmed cases who escaped from healthcare facilities have been reported, one of whom was found deceased in the community in Butembo (a dignified and secure burial was carried out).

South Kivu maintains an active caseload of 5 patients at the Lwiro Ebola Treatment Center (2 confirmed and 3 suspected, including 2 infants receiving formula in a specialized daycare). One admission and two discharges (non-cases) were recorded.

Overall, isolation capacity remains sufficient (71 beds available in North Kivu for 8 patients, occupancy rate of 11%), but the quality of care is compromised by the reported shortage of infection prevention and control (IPC) supplies in North Kivu and by an average IPC score of only 40% in the assessed facilities. The escape of four confirmed cases (1 in Ituri, 3 in North Kivu) poses a major risk of community transmission and the continuation of the epidemic.

4.6 Risk Communication and Community Engagement (RCCE)

Zone of health	Activities	Scope
Bunia	Raising awareness among 43 merchants at the BIGO market about preventive measures, adherence to the EDS and the mode of transmission of EVD.	Large audience
Rwampara	Plea to the Chief of the LENGABO Group to secure his involvement in the EDS activities. This followed attempts by some local youth to disrupt the response activities (RECOS training). 18 community leaders were briefed in the presence of the Governor's advisor's delegate in charge of youth.	190 people
Bamboo	Raising awareness among 10 taxi drivers (including 2 women from the ATAMOI association) about preventive measures and joining the EDS.	10 Men (2 women)
Nizi	Raising awareness about the importance of adhering to Ebola prevention measures, focusing on EDS in the Nizi Centre area/MAMBISA chiefdom	55 people (12 men and 43 women)
Mongwalu	Awareness-raising of chiefs of the 8 districts, avenue chiefs in the presence of the AT, the mayor and the security team, on preventive measures.	55 people (10 women and 45 men)
Aru	Briefing of individuals on CREC and community-based surveillance.	27 people
Nyankunde	Awareness-raising of 31 members of the territorial coordination of the Civil society representatives, including four women, focused on preventative measures and reporting alerts within the community.	31 people including 4 women.

Rumors and misinformation identified

- "Traditional healers have plant-based medicines to cure Ebola virus disease. " • "Previous interventions have not had a positive impact on the ground, which negatively impacts the response interventions on the ground (AIDES, TEAR FUND, OXFAM, UNICEF)."
- " EVD is an abnormal disease, so never put the tarpaulin on our hospital in LENGABO or it will be burned down by the local youth?"
- "Before implementing any activity within our entity, it is imperative to contact the village chiefs, the group leaders, and the AT (Technical Association) and establish a joint committee by appointing a representative CLD, village chief, group leader, PRESICODESA, evaluate the interventions one week before the end of the project.

- Do not schedule polio vaccination during this time of the response for fear that the population will say that their children are being injected with the Ebola virus.

Analysis : These rumors reveal a deep distrust of the healthcare system and response teams, a lack of understanding of treatment procedures, and legitimate economic concerns. A rumor response plan with tailored messaging must be activated.

4.7 Logistics

- Support for the different pillars of the response (provision of transport).
- Continued delivery of motorcycles, UNICEF donation to the ZS.

4.8 Security

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm, but the presence of armed groups in the territories of Djugu, Irumu and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.9 PSEA

- Signing of the code of good conduct for national-level providers on PSEA: 30 men and 08 women were briefed.
- 702 people including 388 men and 310 women and 4 girls in Bunia, Rwampara and Munbgwalu, were briefed and signed the code of conduct commitment documents.

A briefing on PSEA was organized for 52 PoC/PoE providers (32 of whom were women) .
and 20 men).

5. MAIN CHALLENGES

Challenge No.	Impact	Action required
1. Reluctance to perform post-mortem sampling (swabs)	Delay in confirmation; underestimation of cases	Intensive community engagement; involvement of religious leaders
2. Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3. Weakness in contact tracing (overall rate 39.3%, target 95%)	Disruption of transmission chains compromise	Training, deployment and motivation of relays communities
4. Supply failure in Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for the allocation of MEG
5. Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies

6. Low escalation of alerts in the 3 provinces	Delay in detection of case	Strengthening of the community monitoring
7. Spread of rumors and traditional recipes	Distrust of the response; consultation delays	Rumor response plan; social mobilization
8. Insufficient resources financial for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Responsible pillar	Due date
1	Training of service providers on the use of PCR RADIONE	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Official handover of the standardized CTE CME Rwampara to the Incident MVE Manager	Coordination	Day + 1
5	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
6	Catching up on contact tracing in Ituri (current rate 35%)	Monitoring	Day 7
7	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
8	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
9	Catch-up plan for the 47 samples awaiting analysis	Laboratory	Day 2

Supervision of the PoE in Marabo and Irumu, followed by the provision of tools and equipment.



Incident: Red Cross EDS teams attacked at the cemetery.

**FOR ALL
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ADDITIONAL,
PLEASE CONTACT**

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